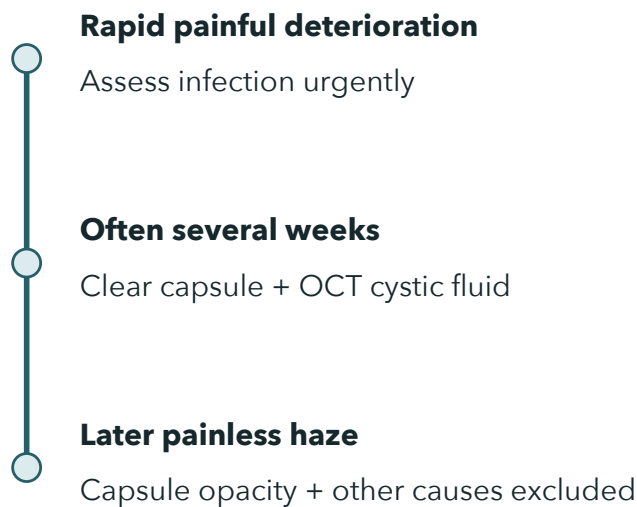


Blur after cataract surgery: localise the cause

The timing helps, but the cornea, capsule, pressure and macula decide.

Use the course as a guide, not proof

A | Three different postoperative patterns



Timing supports; examination determines

Original timeline. Serious infection can occur outside a typical interval; symptoms and examination determine urgency.

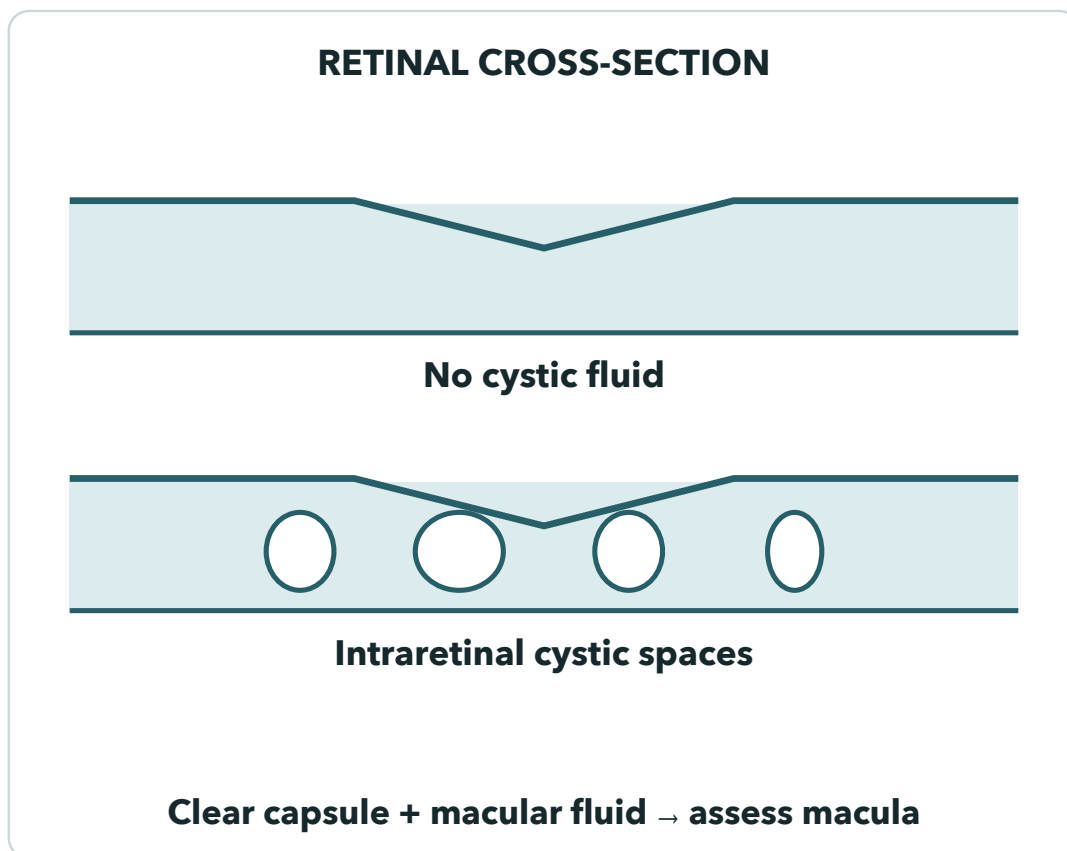
Where is the demonstrated problem?

Evidence	Likely route
Pain; rapid loss; vitritis	Emergency assessment for infection; routine postoperative drops are insufficient.
Weeks; clear capsule; cystic OCT fluid	Consider pseudophakic cystoid macular oedema and retinal review.
Later haze; central capsule opacity	Assess visual significance and exclude retinal or other causes before capsulotomy.
Clear structures; refractive improvement	Consider residual refraction or surface issues after excluding urgent pathology.

References RCOphth | Postoperative macular oedema | AAO EyeWiki | Pseudophakic cystoid macular oedema | NICE NG77 | Cataract recommendations

Retinal fluid is not a cloudy capsule

B | See which layer is abnormal



Original retinal cross-sections illustrating cystic fluid. These are explanatory drawings, not diagnostic OCT scans.

Assess postoperative macular oedema

Cystoid macular oedema often appears several weeks after initially successful surgery. OCT confirms the location of fluid and supports monitoring; pre-existing diabetic disease, uveitis and other retinal causes influence interpretation. Ophthalmology guides anti-inflammatory treatment and escalation when oedema persists, using the local pathway and pressure checks. A clear capsule with macular cysts does not support capsulotomy as the treatment for the blur. New pain, marked redness or sudden deterioration requires urgent reassessment regardless of the scheduled review. A patient who sees well initially, then develops quiet painless blur and macular cysts several weeks later, has a different problem from someone with later capsule opacity. Localising the demonstrated cause avoids applying the same laser solution to every postoperative complaint.